[redacted] False, indusputable

[redacted] False/inaccurate, disputable

[redacted] False, because Gie City lied/withheld information

[redacted] Subjective - projecting their values onto Gie City's life

ACT Government | Canberra Health Services

City Mental Health Team

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FACSIMILE TRANSMISSION

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TO: Dr Randhir Ramchander

DATE: 04/08/2022

FAX NUMBER: 0262418550

NO. OF PAGES (INCLUDING COVER PAGE):

DATE FAXED: 04/08/2022

FROM: City Mental Health Team

URGENT: YES/NO CONFIDENTIAL: YES

RE: Giles Cullity DOB: 22/07/1993

DIAGNOSIS:
Schizoaffective disorder/BPAD
Social anxiety disorder
??ASD

MEDICATIONS:
1. Abilify Maintena LAI 300mg IMI 4 weekly

2. Zopiclone 7.5mg ½ to 1 tablet daily (commenced on 04/08/2022)

Dear Dr Ramchander,

Many thanks for ongoing care for Mr Giles Cullity. I saw Giles in my outpatient clinic today at the City Mental Health Service, Canberra. This was a routine follow up appointment. Giles attended alone for the review.

Details of my today's assessment are as following:

Assessment
2nd review
Attended on time

28 yo single male, nil children, lives in a shared housing through affordable housing program, parents live locally, is on a DSP, and has ndis support in place

Background info (eCR and Giles)
Hx, including forensic Hx noted
Noted Dx of Schizoaffective Disorder/BPAD
??ASD – not formally diagnosed
Hx of being on a CTO (2014-2016) due to non-compliance issues
But at present managed as a voluntary patient

Significant Forensic Hx
Allegedly assaulted police in 2010
Alleged attempted murder in 2013 – Reportedly, whilst homeless, Giles argued with another homeless person over the ownership of a mattress. He then cut the neck of the other person. Received 3-year suspended sentence and maintained on home detention bail with monitoring device on the ankle. Concluded in December 2017. Shows some remorse and victim empathy thought thinks he was provoked by the victim, and he was not mentally well then

Giles gives history of few episodes of 'mild depression' since his teenage years and that he sought counselling then (unable to recall any precipitant though gives history of social phobia (possibly due to underlying undiagnosed ASD) and depression in the context of it (Genetic predisposition to BPAD might explain this too)

One manic episode in 2013 and subsequent admission to a hospital in Adelaide (only admission according to Giles)

Although I note he has had a depressive episode in recent years and was treated with Venlafaxine, which he has self-ceased – but Giles does not recall this episode

Giles has been on Clozapine for over 5 years and has managed to come off gradually due to concerns re to weight (has self-reduced from 300mg nocte over 6 months - been on it since 2017)
Has been on Aripiprazole Depot IMI 300mg every 4 weekly for past 5 years too

At present, Giles has managed to cease Clozapine by gradually reducing it over the past few weeks
He last took 6.75mg on Tuesday

Since further reduction and stopping Clozapine, he has not been able to sleep well
Monday and Tuesday ~3 hours at night
Using sleep hygiene measures
Last night ~1 hour only despite taking Phenergan 25mg nocte and Quetiapine IR 50mg nocte

Lack of sleep is having an impact on his energy levels – 'feeling tired'
Otherwise feeling well

Thinks Quetiapine adding to tiredness, so not keen to continue it too

Noted Over the years has been selective of medication due to the same reason
Does not like meds like Clozapine, Olanzapine, and Mirtazapine Previously ceased Olanzapine (due to weight gain), Citalopram, (due to possible sexual side effects), Paliperidone depot, 75mg slowly reduced to 25mg and then ceased following somatic complaints (?possible sexual side effects), and Risperidone commenced and then ceased following somatic complaints (?possible sexual side effects) – transitioned to Aripiprazole

Strong **family Hx** of BPAD and ASD in younger brother
Paternal grandfather, paternal uncle and maternal uncle - BPAD

Current MS
Giles denies having any decline in MS since reduction/cessation of Clozapine
States feeling 'stable' in mood – 'not high or depressed'
Energy – low in past 3 days due to disturbed sleep; been OK prior to that
Appetite remains good
Routines – exercises for 2 hours daily/ driving lessons with the instructor via ndis, working on gym equipment with ndis worker as well, reading philosophy and stuff related to the gym etc
Enjoys these activities too

Good concentration
Independent with ADLs; 'functioning well'
Visits parents twice a week for dinner; speaks to them daily

Denies having any persecutory or referential ideation
Denies having grandiose beliefs
Denies having thought interference or passivity phenomenon
Denies having AH/VH/OH/TH

States that when unwell; he became "manic and hostile" – nil other delusions
States doing well for past 6-7 years with no psychosis

Though I note from eCR, that there has been a history of somatic delusions as well

On exploration today, Giles states in teenage years, decided to have a 'smart body to impress girls'
Increased preoccupation with the body image since then
Nil ED cognitions
Nil ED behaviours including restriction/binging
Does a lot of gym/exercise/weights etc
Takes high protein diet
Denies any steroid or supplement use ever (though notes indicate that use of large amounts of supplements (creatine and protein powders) have caused hepatic and renal impairment from time to time (elevated CK on repeat bloods and decreased eGFR, GP has been advised)
Says not using them now
But never restricted diet or used compensatory behaviours to lose weight
Does not weigh self at all
Remains conscious of body though – sees mirror after shower once daily before going to bed
Measures biceps girth every day
Wants to keep fit and have a nice figure – admits having increased preoccupation but does not find these thought intrusive or distressing – 'own thoughts'
Some discomfort when not able to exercise
Given these thoughts are ego-syntonic likely not on OCD/body dysmorphia spectrum but on psychotic spectrum (though at present not held with delusional intensity)

Also,
Reports having anxiety – Social Anxiety Disorder from early childhood
But admits to having anxiety in social situations – right from his childhood
'Socially awkward' – issues with communication too
'Shy temperament'

Strong fear of negative evaluation by others
Unsure what triggered it off
Would feel uncomfortable in social situations with somatic Sx but nil frank PA
Would avoid social situations/crowded places/talking and eating in public
Thinks SAD has improved and can eat and speak in public and travel in public transport
Can attend social situations but with extreme discomfort
Nil current somatic Sx/PA
Nil features of GAD

Denies any childhood adversities/trauma
Born in Scotland and moved to Adelaide when 8
Transition went well'; was close to both parents
Father works as professor of Philosophy at ANU – is main support and contact in family. Mother is a retired music teacher
Grew up in Adelaide
2nd of 4 children (all boys)
One younger brother has ASD and one BPAD
States mom would comment him being having anankastic traits as a child – 'perfectionist and would stick to routines'
But does not think he was diagnosed with ASD/ADHD/OCD as a child
Nil behaviour issues at school; attended St Ignatius College but did not enjoy private school environment and then at Marryatville for High School, enjoyed more. Not many friends, never had a girlfriend. Nil bullying and did well academically A's and B's
until year 12 Grades dropped in year 12; Failed year 12 and parents became concerned then

Entered the Uni for a Mechanical Engineering degree but failed first year of bridging course. Became homeless after school to finish his computer project – 'was keener on gym stuff'

Was not able to hold a sustainable job due to decline in MH

Has always struggled to make friends/GF due to social anxiety
Has always been single

Does not think he has any obsessional thoughts/rituals though like to do things in his own way
Class himself a perfectionist
Does come across as somebody with reduced mental flexibility

D&A - non-smoker and denies any regular drug and alcohol use; occasional THC use in school days, non recent
Denies any acute physical health issues

Though alludes to taking some antihypertensive bought online Advised against using online antihypertensive and advised to discuss with GP
Up to date with CMB; due in couple of weeks
Issues around libido secondary to certain meds e.g. Paliperidone

Main concerns re sleep and tiredness
Not keen to take Quetiapine, Olanzapine, or any other antipsychotics

Discussed Asenapin, Latuda, & Cariprazine – not keen on any
Discussed possible increase to Aripiprazole depot to full dose – not keen

Wants to come off depot (vol patient) thinks tablet would help with sleep better than depot
Not keen to continue Phenergan as ineffective
Not taking Melatonin either

Supports
Good family support
NDIS support
Looking for job and working on gym equipment

Risk Ax
Denies having any TOSH/SI/TOHTO
At risk of relapse if non-compliant with the medication
Complying with reviews and depot thus far as a vol patient
At risk of grossly impaired functioning if non-compliant with medication
Moderate to severe risk of harm to others if non-compliant with medication (previous forensic Hx)
Risk to physical health if uses supplements/steroids

Positives – family support, engagement with CMHT and treatment plan on a vol basis, nil active D&A, and good Sx control on treatment

Future – casual work, girlfriend, gym equipment - future focussed and help seeking

MSE
Young male, well build, slightly overgrown shave, casually but appropriately dressed for the day, and maintaining good self-care
Presented well; pleasant and cooperative – good rapport overall, though at times gave some inconsistent account of certain aspect of history
Anxious at the start; progressively relaxed
Mood euthymic with anxious affect
Nil FTD
Nil delusions elicited – though has preoccupation re body image – not held with

delusional intensity at present (as above)
Nil AH/VH
Nil TOSH/SI/TOHTO
Orientated in t/p/p
Displays some insight but at the same time ambivalence re to medication use
Not keen for medication for social anxiety

Impression
29 yo male with a well-established diagnosis of Schizoaffective disorder/BPAD on background of Social Anxiety Disorder and possible? ASD (not formally diagnosed) and perfectionistic traits
Strong family Hx of BPAD and ASD
Significant forensic Hx
Risk assessment as above
Has ceased Clozapine completely now; needs close monitoring of MS given previous forensic Hx – as risk tends to escalate when unwell; though stable MS right now
Nil significant D&A
Good support in place

Agreed Plan

o Psychoeducation provided.
o Discussed at length need for good sleep at night. Reiterated sleep hygiene measures. Discussed and agreed to consider a short-term script for **Zopiclone 7.5mg (1/2 to 1 tab nocte)** to aid sleep – major SE including risk re to long-term use (dependence) discussed. Script provided for 30 tablets and nil repeat (NKDA). Review in 2 weeks.
o Encouraged to **continue Aripiprazole depot** now and review in future. Depot due tomorrow.
o Ongoing FBC monitoring as Clozapine protocol.
o Request for case management; monitoring of MS and risk.
o Request GP for cardiometabolic monitoring + monitoring of kidney and liver functions. Dr Ramchander, If you could kindly organise bloods and ECG and attend to the mental health care plan for psychology, please?
o CM to please chase the echo; Giles states that echo has been done and he has discussed with GP and waiting a review.
o Advised against using online antihypertensive and advised to discuss with the GP.
o Next review with me in 2 weeks' time.
o Giles is aware that he can contact us/emergency services if needed.

Yours Sincerely,

Faisal Yaseen
CMO with Dr Sabir & Dr Rodrigo (Psychiatrists)
The City CMHT, ACT

City Mental Health Team

ACT Government | Canberra Health Services

Level 2, 1 Moore Street, Canberra City ACT 2601

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TO: Dr Randhir Ramchander

DATE: 19/08/2022

FAX NUMBER: 0262418550

NO. OF PAGES (INCLUDING COVER PAGE):

DATE FAXED: 19/08/2022

FROM: City Mental Health Team

URGENT: YES/NO CONFIDENTIAL: YES

RE: Giles Cullity DOB: 22/07/1993

Diagnosis:
Schizoaffective disorder/BPAD
Social anxiety disorder
??ASD

Medications:
1. Abilify Maintena LAI 300mg IMI 4 weekly
2. Zopiclone 3.75mg nocte for a week and then 3.75mg on every alternate night for a week and then cease
3. Olanzapine 20mg nocte

Dear Dr Ramchander,

Many thanks for ongoing care for Mr Giles Cullity. I saw Giles in my outpatient clinic today at the City Mental Health Service, Canberra. This was a routine follow up appointment. Giles attended alone for the review. His new case manager, Venessa Reid (SW) joined us in the review too.

Details of my today's assessment are as following:

Assessment
Follow on review
Attended alone

28 yo single male, nil children, lives in a shared housing through affordable housing program, parents live locally, is on a DSP, and has ndis support in place

Since last review
Commenced Zopiclone that helped with initial insomnia but not with the middle insomnia – found this distressing and commenced Olanzapine 10mg- 20mg nocte after discussion with the CMHT – the combination of Zopiclone and Olanzapine has helped with the sleep – 'up to 8 hours a night'
From past few nights – has reduced Zopiclone to 3.75mg and Olanzapine to 15mg nocte (makes changes himself) - with some disruption again

States MS has remained the same as the last review, however, since he has been on Olanzapine, he has noticed feeling tired with an impact on mood (feeling low intermittently/'not pervasive low mood and not depressed') - reports low energy and some decline in enjoyment too
Though is able to function reasonably well and doing gym routines as before (though has missed a day due to tiredness)
Appetite remains intact
Concentration good
Nil depressive cognition or TOSH/SI/TOHTO
Nil anger/agitation/hostility

Nil hypomanic/manic Sx either

Denies having any persecutory or referential ideation
Denies having grandiose beliefs
Denies having thought interference or passivity phenomenon
Denies having AH/VH/OH/TH

Anxiety – does not believe this has been an issue for him
Although ongoing uncomfortable feelings in social settings and subsequent avoidance - fear of negative evaluation by others

Ongoing concerns re to sleep and think depot is the main culprit
Hence, would like it to cease
Not willing to continue Olanzapine long term due to issues with weight and tiredness, and Zopiclone due to risk of dependence

Ongoing concerns re body image; some increased preoccupation but still not held with delusional intensity

Nil AH/VH
Nil TOSH/SI/TOHTO
Orientated in t/p/p
Displayed some insight but at the same time ambivalence re to medication use
Not keen for medication/psychology for social anxiety

Impression
29 yo male with a well-established diagnosis of Schizoaffective disorder/BPAD on background of Social Anxiety Disorder and possible? ASD (not formally diagnosed) and perfectionistic traits
Strong family Hx of BPAD and ASD
Significant forensic Hx
Risk assessment as above

Has ceased Clozapine completely now; needs close monitoring of MS given previous forensic Hx – as risk tends to escalate when unwell

Psychosis remains in remission but ?emergence of some depressive Sx; need close monitoring

Nil significant D&A
Good support in place

Agreed Plan
o Psychoeducation provided.
o Discussed at length need for good sleep at night. Reiterated sleep hygiene measures. Discussed and agreed to continue **Olanzapine 20mg nocte** for now and reduce **Zopiclone to 3.75mg nocte** this week, then **3.75mg on alternate days for a week and then cease.**
o Agreed to meet up in 2 weeks time to review depot; Giles is keen to switch on to oral antipsychotics. Will review Olanzapine then too.
o Ongoing FBC monitoring as Clozapine protocol. Added Kidney, liver, and thyroid function as well.
o Case management by Vanessa. Monitoring of mental state and risk - emerging depressive Sx. Discussion about antidepressant medications/Lamotrigine to be used in future.
o Request GP for cardiometabolic monitoring + monitoring of kidney and liver functions. Dr Ramchander, If you could kindly organise bloods and ECG and attend to the mental health care plan for psychology, please?
o CM to please chase the echo; Giles states that echo has been done and he has discussed with GP and waiting a review.
o Advised against using online antihypertensive and to discuss with the GP.
o Next review with me in 2 weeks' time.
o Giles is aware that he can contact us/emergency services if needed.

Yours Sincerely,

Faisal Yaseen
CMO with Dr Sabir & Dr Rodrigo (Psychiatrists)
The City CMHT, ACT